SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Atezolizumab + bevacizumab for HCC

First-line advanced or unresectable liver cancer (HCC) · NCCP 00831 · version 2a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This combination aims to control hepatocellular carcinoma, a cancer that starts in the liver, when it is advanced or cannot be removed and no previous systemic treatment has been given. Atezolizumab helps immune cells recognise cancer; bevacizumab blocks a signal that supports blood-vessel growth.

Schedule
Atezolizumab is given first on Day 1 as an IV infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Urgent symptoms and local contact details confirmed
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Support medicines and phase changes explained
- ☐ Common, serious and lasting drug-specific risks discussed below

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Atezolizumab
PD-L1 IMMUNE CHECKPOINT INHIBITOR

- ☐ Immune inflammation of lungs, bowel, liver, kidneys or skin
- ☐ Hormone changes, including thyroid, adrenal or pituitary problems and diabetes; replacement may be long term
- ☐ Rare serious heart, muscle, nerve or brain inflammation
- ☐ Infusion/injection reactions; delayed immune effects after stopping

Bevacizumab
ANTI-VEGF ANTIBODY

- ☐ Raised blood pressure; rarely hypertensive crisis or PRES
- ☐ Protein in urine and kidney injury, including nephrotic syndrome
- ☐ Bleeding, including serious bleeding from varices
- ☐ Arterial or venous blood clots
- ☐ Poor wound healing, bowel perforation or fistula; rare jaw problems
- ☐ Infusion or allergic reactions

Discuss expected benefit and uncertainty, alternatives including not proceeding, liver disease and variceal bleeding, immune toxicity that may be delayed or need lifelong hormone replacement, vascular/kidney/bowel/healing risks, fertility and pregnancy prevention, IV versus thigh administration and your right to reconsider.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00831 v2a.

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